THE WHOLE TREATMENT JOURNEY

HER2 maintenance: treatment and checks continue

Maintenance · HER2-positive metastatic / unresectable recurrent breast cancer

Pertuzumab and trastuzumab continue after the chemotherapy part of the metastatic treatment plan. Maintenance remains active treatment, with response and heart checks.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Earlier combination	Maintenance transition	Every 3 weeks	Regular response review	Beyond year 1
Cancer medicines NCCP 00726	Chemotherapy with HER2 treatment	Review benefit, tolerance and last dose date	Pertuzumab + trastuzumab infusions	Continue while benefit and tolerance support the plan	Ongoing treatment; no fixed one-year stopping rule
Local / endocrine care METASTATIC · EARLY	Prior surgery or radiation varies	Discuss symptom-directed local treatment	Radiotherapy only if separately indicated	Endocrine therapy may be considered if HR-positive	MDT agrees any changes
Blood tests NCCP 00726	Carry previous results forward	Blood count, kidney and liver tests	Before each cycle	Additional tests when indicated	Continue according to the active regimen
Heart ultrasound NCCP 00726 · ESC	Count from first HER2 exposure	Check latest echo and due interval	Symptoms reviewed every visit	NCCP: echo every 12 weeks	12-weekly default; interval changes require cardiac risk review
Cancer scans METASTATIC	Baseline extent documented	Use prior scans as comparison	Report new symptoms between scans	Usually every 2–4 months; modality / interval individualised	Keep a documented response-assessment plan
Tumour markers MARKERS	Consider if informative	Confirm whether used for this person	No universal marker schedule	Adjunct to symptoms and scans, never alone	Continue only when clinically useful

What each check is for

1 Heart ultrasound

NCCP: baseline and every 12 weeks during HER2 treatment

Checks heart function while treatment continues.

Ask / act: Ask when the next echo is due; a move to maintenance does not restart the clock.

Sources: NCCP 00726 · ESC

2 Response imaging

Usually every 2–4 months; earlier if clinically needed

A baseline scan records where the cancer is. Response is usually reviewed every 2–4 months, adjusted for the treatment, disease pace, symptoms and previous results. CT chest/abdomen/pelvis is common when appropriate; the team chooses the imaging needed for the known sites.

Ask / act: Ask what scan is planned, when results will be discussed and who to contact for new symptoms.

Sources: METASTATIC

3 Tumour markers, if useful

Only if included in the individual monitoring plan

CA 15-3 or CEA may be helpful in selected metastatic cases, especially if previously raised. They support the assessment with symptoms and imaging; a marker alone does not decide whether treatment is working or should change.

Ask / act: Ask what a change means alongside the scan and clinical review.

Sources: MARKERS

4 Missed or delayed infusion

Whenever a treatment gap occurs

A long gap can change the required antibody loading doses.

Ask / act: Contact the team to confirm the new prescription; do not assume the old visit plan still applies.

Sources: NCCP 00726

5

Separate early-breast pathways

At every transition

This particular NCCP code is for metastatic/unresectable recurrent maintenance.

Ask / act: Ask the team to confirm the setting and aim on the written plan.

Sources: NCCP 00726

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.